A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

18 May 2026

Dr Leila Byrne
Turn The Corner Medical Clinic

Dear Dr Byrne,

Re: Ms Holly Mortlock
DOB: 02/07/1990 Phone: 0403 840 945
Address: 1/107 Hope Street, BRUNSWICK, VIC, 3056
Email: hm.mortie@icloud.com

Summary

- **Significant improvement in eosinophilic oesophagitis (EoE) symptoms with expanded dietary tolerance.**
- **Stable swallow following self-discontinuation of PPI therapy.**
- **Debilitating systemic fatigue and diaphragmatic pressure of unclear etiology.**
- **Blood tests arranged to exclude rare autoimmune and neurological conditions.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Holly. She reports a pleasing improvement in her symptoms of eosinophilic oesophagitis. Her oesophageal pain resolved first, followed by a gradual improvement in her swallowing function. She has progressed from a "mushy" diet to being able to tolerate raw cucumber and apple.

Interestingly, Holly has been off her PPI for approximately a week. As she has not noticed any recurrence of chest pain or dysphagia, I have suggested she remains off this for the time being. We always have the option to restart therapy should her symptoms fluctuate. Her weight remains relatively stable between 65 and 67 kg. Her bowel habit continues to fluctuate between weeks of constipation and more regular movements, which she is currently tracking against her menstrual cycle.

The primary concern at present is a fatigue. This appears to be centered around what Holly perceives to be her diaphragm and is severe enough to restrict her ability to speak at times. She describes it as a weight or pressure that makes the act of talking more exhausting than physical exertion. She also reports blurry vision with "ghosting" of text and postural dizziness when rising from a chair or bed. Furthermore, she has noted a specific "crash" of extreme sleepiness following carbohydrate intake, which may be related to her previous adherence to a ketogenic diet.

Impression and Plan While it is possible these systemic symptoms represent a hypersensitivity state or an amplification of various physiological factors which i have outlined in previous letters, I believe it is prudent to exclude formal pathology. I have therefore ordered a comprehensive battery of blood tests. In addition to monitoring her iron stores and B12 levels—noting that her recent B12 result was expectedly high following injections—I am testing for several rare autoimmune and neurological disorders. Ultimately I think this is

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
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Hobson Healthcare Sydenham
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St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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unlikely I have stated this to Holly.

If these tests are negative, we will have more confidence that we are not missing a primary neurological or autoimmune process. I have suggested that a neurology referral may be a useful next step depending on these results. I will review Holly in approximately one month to discuss the findings. In the interim, I have advised her that I can provide repeat scripts for her gastrointestinal medications via email if required.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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